CENTERLINE

PILATES

VOLUME 7

The Instructor Manual

Fundamentals, anatomy, teaching guides and modifications

THE PILATES INSTRUCTOR'S LIBRARY

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Welcome to the Library

> *This manual is the seventh and final volume of The Pilates Instructor's Library. It is the foundation under the other six: the principles, the anatomy, the teaching craft, the safety rules and the language that make 500 exercises and 100 classes work in a real studio with real people.*

Read it once from start to finish, then keep it close. Most instructors return to the Cue Bank before a class, the Modifications chapter when a new client mentions an injury, and the Glossary when a colleague uses a term they have not met. Everything here is written to be used, not admired.

How the seven volumes fit together

VOLUME	WHAT IT CONTAINS	WHEN TO OPEN IT
Volume 1: Beginner Exercises	170 illustrated exercises for new clients and foundational work	Planning first sessions, teaching basics, finding a regression
Volume 2: Intermediate Exercises	170 exercises that add range, coordination and load	Progressing a client who owns the basics
Volume 3: Advanced Exercises	160 exercises with high demand on control and strength	Experienced clients, athletes, your own practice
Volume 4: Beginner Classes	Classes 1 to 40, each 60 minutes	Group classes for mixed or new groups
Volume 5: Intermediate Classes	Classes 41 to 70, each 60 minutes	Regulars who are ready for more challenge
Volume 6: Advanced Classes	Classes 71 to 100, each 60 minutes	Experienced groups and workshops
Volume 7: The Instructor Manual	Principles, breathing, anatomy, equipment, teaching skills, modifications, safety, glossary, cue bank	Always. This is your reference layer

Together the exercise volumes hold 500 exercises: 170 beginner, 170 intermediate and 160 advanced. The class volumes hold 100 complete 60-minute classes built from those exercises. Nothing in a class is a mystery; every movement named in a class plan can be looked up in an exercise volume.

Exercises are organized by level

Each exercise carries a level label so you can match it to the person in front of you. Levels describe the demand on control, strength, mobility and coordination, not the worth of the client. A beginner exercise done with precision is better teaching than an advanced exercise done sloppily.

- **Beginner:** simple positions, small ranges, one thing to think about at a time. Builds breath, alignment and awareness.
- **Intermediate:** longer levers, more transitions, light coordination of arms and legs, moderate spring or ball challenge.
- **Advanced:** large ranges, minimal support, complex sequencing and fast changes of position. Assumes strong foundations and no relevant injury.

Classes are organized by level too

Classes 1 to 40 are beginner, 41 to 70 intermediate, 71 to 100 advanced. Each is a full 60 minutes with a warm-up, a main block and a cool-down. You can teach a class exactly as written, swap a block for another of the same category, or use it as a template for your own design. Section 8 shows you how to build a class in five minutes from the same building blocks.

How to read an exercise card

Every exercise follows the same layout so you can scan it in seconds, even mid-class. Learn the layout once and the whole library opens up.

FIELD	WHAT IT TELLS YOU	HOW TO USE IT
Level and equipment	Beginner, intermediate or advanced; Mat, Band, Magic Circle, Stability Ball, Wall, Reformer, Cadillac, Wunda Chair or Barrel	Filter for what your room and client allow
Focus	The main intention of the exercise, such as spinal articulation or hip stability	Tells you why you are choosing it
Target muscles	The muscles doing most of the work	Balance a class so no region is ignored
Springs	Typical spring setup on spring-based equipment, in plain language	Starting point only; adjust to the client
Dosage	Reps, seconds or breaths, and sets	Plan your timing; each exercise lists minutes
Breathing	When to inhale and when to exhale	Teach the breath first, then add movement

FIELD	WHAT IT TELLS YOU	HOW TO USE IT
Common faults	What usually goes wrong	Know what to watch for before you cue
Progression / Regression	A harder and an easier version	Adjust in the moment without stopping the class
Cue	The exact sentence to say aloud	Say it as written, then adapt to your voice
Adaptation	A safe alternative for common limitations	Offer it early, never as a punishment
Steps	Starting position and movement, in order	Learn the exercise before you teach it

How to read a class

A class plan lists its blocks in order with a time for each, the exercises in each block, the equipment you need, and a short teaching note. Times add up to 60 minutes. The structure is always the same: roughly 10 minutes of warm-up, 40 minutes of main work and 10 minutes of cool-down. Read the full class once before teaching it, rehearse the transitions out loud, and set up the room before clients arrive.

> **Educational use only**
>
> This library is an educational resource for certified Pilates instructors and instructors in training. It is not medical advice and does not replace training, supervision or the guidance of a client's healthcare professional. Use your judgement and your scope of practice at all times.

BEFORE YOU TEACH ANY CLASS FROM THIS LIBRARY

☐ Read the whole class plan, including the teaching notes.

☐ Check the client intake and any known limitations.

☐ Set up the equipment and test springs, straps and footbars.

☐ Choose your modification for each block in advance.

☐ Know your first and last cue.

The Six Principles

> *The six principles are the common thread through every exercise and every class. They are not decoration or history; they are a teaching checklist. When a class*

> *feels off, one of them is usually missing.*

Different lineages name and order them differently. This manual uses six: concentration, control, centre, flow, precision and breath. Treat them as lenses you hold up to the work, not as rules to recite.

1. Concentration

Concentration means the client's attention is in their body. It is the difference between moving and exercising mindfully. As an instructor you cannot force it, but you can protect it: give one cue at a time, keep your language simple and avoid chatter during demanding work.

- **In practice:** ask clients to feel one specific thing, such as the ribs resting down or the weight on the heels.
- **Teaching tip:** use silence. A pause of three breaths lets people find the movement themselves.
- **Sign it is missing:** eyes wandering, fast sloppy reps, clients talking through the exercise.

2. Control

Control means every movement is chosen, not thrown. Slower is usually easier to see and harder to do. Control is also what protects joints, because momentum hides weakness and places load where the body is not ready.

- **In practice:** count the tempo out loud, such as 'three counts out, three counts back'.
- **Teaching tip:** reward the slowest, cleanest rep, not the biggest.
- **Sign it is missing:** bouncing, jerking, springs slamming, limbs dropping at the end of a range.

3. Centre

The centre, often called the powerhouse, is the area between the lower ribs and the pelvis, including the deep abdominals, the pelvic floor, the deep back muscles and the hips. Movement is meant to start from here and spread outward. A client with a stable centre can move arms and legs with less strain on the neck and lower back.

- **In practice:** begin each exercise by finding a gentle engagement of the deep abdominals before the limbs move.
- **Teaching tip:** ask for 20 to 30 percent effort, not a maximal brace. Over-gripping steals breath and ruins flow.
- **Sign it is missing:** the ribs flare, the back arches, the neck takes over.

4. Flow

Flow is smooth, continuous movement with a natural rhythm. It does not mean fast. A slow exercise can flow; a rushed one rarely does. Flow also applies to the class as a whole: transitions between positions should feel like part of the work.

- **In practice:** link breath and movement so each phase of an exercise has a clear start and finish.
- **Teaching tip:** plan your transitions as carefully as your exercises.
- **Sign it is missing:** stop-start movement, long pauses to reposition, clients lost between exercises.

5. Precision

Precision means every exercise is done the right way, with attention to alignment and detail. It is better to do five accurate reps than twelve vague ones. Precision does not mean perfection; it means honest effort toward a clear target.

- **In practice:** pick one or two details per exercise and teach those well.
- **Teaching tip:** correct the largest fault first and ignore the smaller ones until the big one is fixed.
- **Sign it is missing:** the shape of the exercise changes from rep to rep.

6. Breath

Breath links the principles. It sets rhythm, helps the deep abdominals engage, and keeps the nervous system calm enough to learn. Section 3 covers breathing in detail. For now, remember that a client who holds their breath is not ready for the next progression.

Using the principles to teach

When you plan or watch a class, run a quick check against the six principles. It gives you a vocabulary for what you see and a simple order for what to fix.

PRINCIPLE	QUESTION TO ASK	TYPICAL FIX
Concentration	Is the client's attention on the movement?	Reduce cues, slow down, use a clearer single image
Control	Is the movement chosen or thrown?	Shorter range, slower tempo, lighter springs
Centre	Does the movement start from the centre?	Return to a simpler position and re-find the deep abdominals

PRINCIPLE	QUESTION TO ASK	TYPICAL FIX
Flow	Does it move smoothly from phase to phase?	Tie breath to movement, trim extra cues
Precision	Is the shape consistent?	Fewer reps, one detail at a time
Breath	Is the client breathing freely?	Count breaths instead of reps, cue the exhale

> **A note on teaching language**
>
> Avoid telling clients they are doing something 'wrong'. Say what you want to see instead. 'Let the ribs soften down' lands better than 'stop flaring your ribs'.

Breathing

> *Breath is the first thing a Pilates client learns and the last thing they master. Good breathing supports the deep abdominals, frees the ribcage and gives every exercise its rhythm.*

Lateral thoracic breathing

Pilates breathing is usually taught as lateral thoracic (also called ribcage or intercostal) breathing. The aim is to expand the ribcage sideways and back on the inhale while keeping the deep abdominals gently engaged, so the abdominal wall stays supported through the whole breath cycle.

This is different from belly breathing, in which the abdomen pushes out on the inhale. Belly breathing is useful for relaxation and has its place, but it is difficult to combine with abdominal engagement during movement.

How to teach it

1. Have the client lie on their back with knees bent, or sit tall. Rest the hands on the sides of the lower ribs.
2. Ask them to breathe in through the nose and feel the ribs widen into the hands, like an umbrella opening.
3. Ask them to breathe out through pursed lips, feeling the ribs close and the lower abdominals draw in and up.
4. Keep the shoulders relaxed and the neck long. The upper chest should not heave.
5. Practice for five to eight breaths, then repeat with a light band or towel around the ribs for tactile feedback.

> **Comfort first**
>
> If a client feels dizzy, tingly or anxious during breath work, return to normal breathing and rest. Breath work should never feel forced.

Breath patterns by exercise type

Most exercises follow one of a few patterns. Learn them and you can teach breathing for exercises you have never seen before.

EXERCISE TYPE	USUAL PATTERN	REASON
Flexion (curl up, roll up)	Inhale to prepare, exhale to curl	Exhale supports abdominal contraction
Extension (swan, back extension)	Inhale to lengthen, exhale to lift or return depending on exercise	Keeps the ribs free and avoids compression
Rotation (spine twist, criss-cross)	Inhale to lengthen, exhale to rotate	Exhale deepens the twist safely
Lateral flexion (side bend)	Inhale to prepare, exhale to reach over	Opens the ribs on the long side
Leg or arm work with stable trunk	Inhale to prepare, exhale on the effort	Keeps centre engaged on the hardest part
Pulsing or beating work (hundred, scissors)	Short inhales and exhales in a steady count	Maintains rhythm and avoids holding
Stretches and cool-down	Slow, full breath with long exhale	Calms the nervous system
Balance exercises	Steady, continuous breath	Holding the breath destabilizes

These are defaults, not laws. Some exercises are taught with the opposite pattern depending on tradition or the aim of the lesson. What matters is that you are consistent, you can explain your choice, and the client is breathing.

Breathing cues that work

- 'Breathe into the sides of your ribs.'
- 'Exhale like you are fogging a mirror.'
- 'Let the breath guide the movement, not the other way around.'
- 'Inhale to prepare, exhale to move.'
- 'Keep breathing; don't hold anything.'

Common breathing problems

PROBLEM	WHAT YOU SEE	WHAT TO TRY
Breath holding	Red face, tight jaw, stopped movement	Count breaths aloud, lighten the load
Upper chest breathing	Shoulders rise on the inhale	Hands on ribs, cue 'wide, not up'
Belly gripping	Abdomen hard and stiff, short breath	Reduce effort to 20 to 30 percent
Reverse breathing	Abdomen rises on exhale	Return to supine, practice slowly
Over-breathing	Fast, noisy breaths	Slow the tempo, lengthen the exhale

> **Cue the exhale, not the inhale**
>
> Clients usually know how to inhale. The exhale is where the work happens, so most of your breath cues should land there.

Posture and Alignment

> *Alignment is the map you use to see a client. Before you can cue, correct or modify, you need a clear picture of what good organization looks like and how much natural variation is normal.*

There is no single perfect posture. Bodies differ in bone shape, joint range and habits. The goal is not to force everyone into a textbook line but to find the most efficient, comfortable and strong organization for each person, and to teach them to notice it.

Neutral spine

Neutral spine is the position in which the natural curves of the spine (a gentle inward curve in the neck, an outward curve in the upper back, an inward curve in the lower back) are present and the pelvis is level. In supine it is often described as the pelvis sitting between a flat-back position (imprint) and a full arch.

- **Finding it:** from lying, rock the pelvis gently toward an arch, then toward a flat back, then settle halfway between.
- **Checking it:** on a client lying down with knees bent, the front hip bones and the pubic bone should sit roughly level, like a bowl that is not spilling forward or back.
- **Why it matters:** neutral places the spine in a position where it can share load well and where the deep muscles can work.

> **Neutral is individual**
>
> Do not insist on a textbook neutral. Some clients have a larger or smaller lumbar curve by nature. Aim for a comfortable, stable position where they can breathe and the ribs are quiet.

Imprint

Imprint is a small posterior tilt of the pelvis that brings the lower back gently toward the mat. It is used in some exercises, especially when the legs are lifted, to protect the lower back and encourage abdominal control. It should be subtle: a soft drawing in, not a hard squeeze or a pushed-down back.

- **Cue:** 'Let your lower back melt toward the mat.'
- **Avoid:** gripping the buttocks, lifting the tailbone high, flattening the back with force.
- **Use it when:** the legs are in tabletop or extended, and the client's back lifts off the mat.

The pelvic clock

The pelvic clock is a simple imagery tool for teaching pelvic control. Imagine a clock face on the lower abdomen with twelve at the navel and six at the pubic bone. The client rolls the pelvis toward twelve (posterior tilt), toward six (anterior tilt), then toward three and nine (side to side).

1. Lie supine with knees bent and feet flat, hip-width apart.
2. Rock the pelvis toward twelve o'clock, flattening the lower back, then toward six.
3. Move to three and nine, feeling the weight shift from one sitting bone to the other.
4. Combine them into a slow circle, one direction then the other.
5. Finish by finding neutral and resting.

The pelvic clock is excellent for beginners, for clients with stiff backs, and as a short assessment: you will see quickly who can move the pelvis independently from the ribs and legs.

Scapulae and shoulder girdle

The shoulder blades sit on the back of the ribcage and should be able to glide, rotate and tilt as the arms move. In many exercises the aim is stable shoulder blades that are neither pinched together nor wildly winged. Think of them as resting wide and heavy on the back.

- **Common faults:** shrugged shoulders, winged shoulder blades, shoulders rolled forward, over-squeezing the shoulder blades together.
- **Useful cues:** 'Slide your shoulder blades into your back pockets.' 'Widen across your collarbones.'
- **Why it matters:** poor shoulder blade control overloads the neck and the shoulder joint.

Neck and head

The head is heavy, about the weight of a bowling ball, and the neck muscles tire quickly. In every exercise where the head lifts, ask whether the client can keep the neck long and the chin soft. If not, lower the head and shorten the work.

- **Neutral neck:** ears over shoulders, eyes forward, back of the neck long.
- **Chin position:** a small nod, as if holding a tennis ball under the chin. Not tucked hard, not poking forward.
- **Common faults:** chin jutting forward when curling up, neck cranking, jaw clenching.
- **Fix:** rest the head back, use a pillow or hands behind the head, or shorten the range.

Ribcage

Ribs that flare upward and forward are a common sign that the abdominals are not supporting the trunk. Cue the lower ribs to soften toward the pelvis without collapsing the chest. This is especially important in extension, overhead arm work and leg work in supine.

A quick visual check

REGION	LOOK FOR	IF IT IS OFF
Head and neck	Long back of the neck, eyes level	Support with a pillow, lower the head
Shoulders	Level, relaxed, away from the ears	Cue widening, reduce load
Ribs	Quiet, stacked over the pelvis	Cue ribs to soften down
Pelvis	Level, neutral or intentionally tilted	Return to pelvic clock
Knees	Tracking over the middle toes	Check foot position, reduce range
Feet	Weight spread across the whole foot	Cue three points of contact

ALIGNMENT SCAN BEFORE YOU CUE

☐ Is the client breathing freely?

☐ Is the pelvis in the position the exercise needs?

☐ Are the ribs quiet?

☐ Are the shoulders away from the ears?

☐ Is the neck long?

☐ Is one change enough, or do you need to modify the exercise?

Anatomy for Pilates Instructors

> *You do not need to be an anatomist to teach Pilates well, but you do need a working vocabulary of muscles and joints. This chapter keeps the language clear and practical: what each region does, which muscles matter most, and what that means in a class.*

Educational information only

Anatomy descriptions here are simplified for teaching. They do not diagnose, treat or replace the guidance of a physician or physical therapist.

Core and abdominals

The core is not a single muscle. It is a group that stabilizes the trunk and transfers force between the upper and lower body. Think of it as a cylinder with a diaphragm on top, a pelvic floor at the bottom, deep abdominals at the front and sides, and deep back muscles behind.

MUSCLE	WHERE IT IS	WHAT IT DOES	CLASS EXAMPLE
Transversus abdominis	Deepest layer, wraps the trunk like a corset	Stabilizes the trunk and draws the abdomen in	Hollowing in all four-point work
Rectus abdominis	Front, the 'six-pack' muscle	Flexes the spine, curls the ribs toward the pelvis	Curl up, Hundred
Internal obliques	Sides, under the external	Rotate and side-bend, assist stability	Criss-cross
External obliques	Sides, outermost	Rotate the trunk, resist rotation	Spine twist, Saw
Pelvic floor	Base of the pelvis	Supports organs, assists trunk stability	Pelvic tilt with exhale

MUSCLE	WHERE IT IS	WHAT IT DOES	CLASS EXAMPLE
Diaphragm	Under the lungs	Main breathing muscle	Every exercise
Multifidus	Deep, along the spine	Fine control of each vertebra	Spine stretch, swimming

The spine

The spine has 24 movable vertebrae stacked in three regions plus the sacrum and coccyx. Pilates trains all of its movements: flexion, extension, side bending and rotation. The aim is to make each region mobile where it should be and stable where it must be.

REGION	NUMBER OF VERTEBRAE	MAIN JOB	NOTES FOR TEACHING
Cervical (neck)	7	Mobility, head support	Keep it long, avoid cranking
Thoracic (upper and mid back)	12	Rotation and extension with the ribs	Often stiff; benefits from mobility work
Lumbar (lower back)	5	Weight bearing, flexion and extension	Needs stability more than range
Sacrum and coccyx	5 fused plus a few small	Connect spine to pelvis	Weight bearing in sitting

- **Flexion:** rounding forward, as in a roll up.
- **Extension:** arching back, as in a swan.
- **Lateral flexion:** side bending, as in a mermaid.
- **Rotation:** twisting, as in a spine twist.

A useful rule: the thoracic spine rotates best, the lumbar spine rotates little, and the neck follows the upper back. Cue rotation from the ribs, not from the lower back.

The pelvis

The pelvis links the spine to the legs. It is made of two hip bones and the sacrum, joined at the pubic symphysis and the sacroiliac joints. Pelvic position strongly influences lumbar spine posture, so many cues in Pilates are really cues about the pelvis.

- **Anterior tilt:** the front of the pelvis tips down, increasing the lumbar curve.
- **Posterior tilt:** the front tips up, flattening the lumbar curve.
- **Lateral tilt:** one hip higher than the other.

- **Rotation:** one side of the pelvis moves forward.

The hip

The hip is a ball-and-socket joint built for stability and power. It moves in all directions: flexion, extension, abduction, adduction, and internal and external rotation. A great deal of Pilates is about moving the legs without moving the pelvis.

MUSCLE GROUP	KEY MUSCLES	FUNCTION	TEACHING NOTE
Hip flexors	Iliopsoas, rectus femoris	Lift the thigh toward the trunk	Often tight; avoid gripping in curls
Gluteals	Gluteus maximus, medius, minimus	Extend, abduct and stabilize the hip	Often underused; key for pelvic control
Hamstrings	Biceps femoris, semitendinosus, semimembranosus	Extend the hip, bend the knee	Stretch gently; do not force range
Adductors	Adductor longus, brevis, magnus, gracilis	Draw the thigh toward the midline	Great for inner thigh and pelvic stability
Deep rotators	Piriformis and others	Rotate the hip, stabilize the joint	Tight in many seated workers
Abductors	Gluteus medius, tensor fasciae latae	Lift the leg to the side, stabilize pelvis	Side-lying work

The shoulder

The shoulder is the most mobile joint in the body and therefore the least stable. It relies heavily on the muscles around the shoulder blade and the rotator cuff to stay in place.

MUSCLE	FUNCTION	CLASS EXAMPLE
Rotator cuff (supraspinatus, infraspinatus, teres minor, subscapularis)	Centres the ball in the socket	Arm circles, external rotation with band
Deltoids	Lift the arm in all directions	Arm series
Serratus anterior	Holds the shoulder blade against the ribs	Plank, push up
Trapezius	Moves and stabilizes the shoulder blade and neck	Swimming, rowing

MUSCLE	FUNCTION	CLASS EXAMPLE
Rhomboids	Draw the shoulder blades together	Pulling arms
Latissimus dorsi	Pulls the arm down and back	Pulling straps, rolling like a ball with arms
Pectoralis major and minor	Draw the arm across the chest, tilt the shoulder blade	Chest expansion, push up

The knee

The knee is a hinge joint with a small amount of rotation. It is guided by the hip above and the foot below, so knee pain often starts elsewhere. The knee should track over the middle of the foot, not collapse inward.

- **Quadriceps:** straighten the knee and flex the hip.
- **Hamstrings:** bend the knee and extend the hip.
- **Popliteus:** unlocks the knee.
- **Teaching note:** avoid hyperextending the knee; keep a micro-bend in standing work and cue 'soft knees'.

The foot and ankle

The foot is a spring and a base. A strong arch and active toes help the whole leg. Many clients have stiff or weak feet and benefit from foot work as warm-up and as a conscious part of standing work.

- **Three points of contact:** the base of the big toe, the base of the little toe, and the heel.
- **Intrinsic muscles:** small muscles within the foot that support the arch.
- **Calf muscles:** gastrocnemius and soleus, which point the foot and drive footwork on the reformer.
- **Tibialis anterior:** lifts the foot and controls its lowering.

Anatomy at a glance for class design

REGION	MOBILITY GOAL	STABILITY GOAL
Neck	Gentle rotation and nod	Hold the head with the deep neck flexors
Upper back	Extension and rotation	Hold shape under arm load
Lower back	Small, controlled movement	Resist arching and rotating

REGION	MOBILITY GOAL	STABILITY GOAL
Pelvis	Independent tilts	Hold level during leg work
Hip	Full range in all directions	Keep the pelvis still while the leg moves
Shoulder	Overhead reach, rotation	Keep the shoulder blade stable
Knee	Smooth bend and straighten	Track over the toes
Foot	Articulating through the arch	Spread weight evenly

Equipment Guide

> *Pilates equipment can be as simple as a mat or as intricate as a Cadillac. Every piece has a purpose, a set of safety checks and a way to scale load up or down. This chapter covers the nine pieces used throughout the library.*

> **Always follow the manufacturer's instructions**
>
> Equipment varies between manufacturers and models. Use this guide as general orientation only and defer to the manufacturer's manual for setup, weight limits and maintenance.

Mat

- **Purpose:** the foundation for classical and contemporary mat work, with no equipment required.
- **Setup:** a firm, non-slip mat, about 1 to 1.5 centimetres thick, long enough for the spine and head. Add a folded towel for the head or a pillow where needed.
- **Safety:** avoid slippery floors; check that mats are not worn flat; keep the area around the mat clear.
- **Maintenance:** wipe down after each class with a gentle cleaner; air dry; store flat or rolled.

Band (resistance band)

- **Purpose:** adds light resistance, feedback and stretching assistance.
- **Setup:** choose a resistance level from light to heavy and check for tears before each use.
- **Safety:** hold the band firmly, avoid snapping it back, and never use it if it is cracked or thinning.
- **Maintenance:** keep out of the sun, dust with talc if sticky, replace regularly.

Magic Circle

- **Purpose:** a flexible ring that provides light resistance for the inner and outer thighs, arms and chest.
- **Setup:** hold between the hands, ankles or thighs; use the padded areas for contact.
- **Safety:** do not overload or bend beyond its range; watch for slipping from sweaty hands or feet.
- **Maintenance:** inspect pads and the frame for cracks; store flat.

Stability Ball

- **Purpose:** challenges balance and trunk control, supports the spine in stretches, and adds fun to the class.
- **Setup:** choose the size so that, when seated, hips and knees are roughly level. Common sizes run from about 55 to 75 centimetres across; the client's height guides the choice.
- **Inflation:** firm but with slight give; over-inflated balls are slippery and hard to control.
- **Safety:** use a non-slip floor, keep sharp objects away and never let clients bounce on it unguided.
- **Maintenance:** check for leaks, wipe clean, store out of direct heat.

Wall (Wall Pilates)

- **Purpose:** uses a wall for feedback, support and light resistance, ideal for beginners and home practice.
- **Setup:** clear a flat, sturdy wall free of pictures and shelves; the floor should be non-slip; clients wear socks with grips or go barefoot.
- **Safety:** keep a safe distance between clients; never push off a wall that is not solid; avoid forceful pushing.
- **Maintenance:** wipe marks, keep the floor clean.

Reformer

The reformer is a sliding carriage on rails, with adjustable springs, a footbar, straps and a headrest. It supports and challenges the body in many positions. Springs provide resistance, and the carriage's movement trains control.

- **Springs:** heavier springs make the carriage harder to move out but easier to control on the return in many exercises; lighter springs make it feel freer and less

stable. A common starting point for footwork is three to four springs, in a mix of heavier and lighter.

- **Footbar:** set to the client's height and the exercise; always check it is locked.
- **Headrest and shoulder blocks:** raise or lower for neck comfort and set properly for lying exercises.
- **Straps:** adjust length so hands or feet reach the correct position; check ropes for fraying.
- **Safety:** always check spring attachments, never leave the carriage unguided under load, teach getting on and off carefully, keep hands clear of the spring and carriage gaps.
- **Maintenance:** clean rails and wheels with a soft cloth, check upholstery and ropes, replace worn springs as the manufacturer advises.

Cadillac (Trapeze Table)

- **Purpose:** a raised table with a frame, bars, springs and straps that supports a huge variety of exercises, including hanging and assisted work.
- **Springs:** push-through bars, leg springs and arm springs, each with a range of heavier to lighter. Start light and add as control grows.
- **Safety:** check that bars are locked, springs attached, and straps sound. Keep clients away from the frame edges and pinch points. Spot all inverted or hanging work.
- **Maintenance:** wipe frame and mat, check hardware for tightness, inspect springs and straps.

Wunda Chair

- **Purpose:** a compact apparatus with a pedal and seat that challenges balance, leg strength and trunk control.
- **Springs:** adjust the tension of the pedal; heavier springs make the pedal harder to push, and often more stable for beginners in some exercises, while lighter springs demand more control.
- **Safety:** spot clients on and off the pedal, stand in a safe position, and use the handles for support when needed.
- **Maintenance:** check pedal hinges, springs and any attached handles.

Barrel (Ladder Barrel and Spine Corrector)

- **Purpose:** a curved surface that supports spinal extension, stretches and lateral work.

- **Setup:** use the ladder barrel with the feet and hands on the rungs, or a spine corrector as a smaller shaped surface; adjust to the client's size.
- **Safety:** secure it on the floor, use a spotter for mounting, and check for stability.
- **Maintenance:** wipe the surface and inspect the base.

Springs and tension at a glance

EQUIPMENT	TYPICAL LOAD LANGUAGE	RULE OF THUMB
Reformer	Heavier springs / lighter springs, usually 1 to 4 springs	More springs for stability and footwork; fewer for control challenge
Cadillac	Heavier or lighter springs on push-through bar and straps	Start light and increase gradually
Wunda Chair	Heavier or lighter pedal tension	Start with more support for beginners
Barrel	Usually no springs, adjustable position	Adjust height and distance
Mat and small props	No springs	Adjust band strength or ball size

Spring weights vary between manufacturers and the same colour can mean different things. This library describes springs as heavier or lighter along with a typical count rather than by colour. Learn how the equipment in your studio feels and write your own notes.

PRE-CLASS EQUIPMENT SAFETY CHECK

☐ Springs attached firmly and not stretched or damaged.

☐ Footbar, pedals and bars locked in the right position.

☐ Straps and ropes free from fraying.

☐ Headrests and shoulder blocks secure.

☐ Mats and floor clear and non-slip.

☐ Each client oriented to getting on and off safely.

How to Teach

> *Knowing the exercises is half the job. The other half is the craft of teaching: structuring a class, pacing it, cueing clearly, correcting kindly and holding the room with your voice and presence.*

Structure of a 60-minute class

Every class in this library follows the same simple shape: a warm-up, a main block, and a cool-down. The proportions are 10 minutes, 40 minutes and 10 minutes. The shape is flexible, but it has a logic that protects clients and gives them a satisfying experience.

PHASE	TIME	PURPOSE	TYPICAL CATEGORIES
Warm-up	About 10 min	Arrive, breathe, mobilize, prepare the centre	breath, warmup, spine
Main block	About 40 min	Strength, control, coordination and challenge	abs, legs, arms, full, balance
Cool-down	About 10 min	Stretch, release, down-regulate, close	stretch, cooldown, breath

The warm-up (10 minutes)

- Start with breath and awareness for 2 to 3 minutes. Let the room settle.
- Add gentle spinal movement such as pelvic clock, cat and cow, or a small roll down.
- Introduce the first activation of the centre.
- Include a joint warm-up for the day's focus, such as hips for a leg-heavy class.
- End the warm-up with one exercise that previews the main work.

The main block (40 minutes)

- Begin with the most demanding trunk work while energy is fresh.
- Alternate regions so no area is overworked: abs, then legs, then arms, then full-body.
- Insert a balance or coordination challenge in the middle.
- Include a counter-movement after flexion-heavy work, such as a gentle extension.
- Finish the block with one integrative full-body exercise.

The cool-down (10 minutes)

- Slow the pace and lower the voice.
- Stretch the areas worked, holding for 30 seconds or about three breaths.
- Reconnect to the breath and the sensation of the body.
- Finish with one clear takeaway, such as a gentle reminder of the day's principle.
- Give people a moment of silence before they leave.

Pacing

Pacing is the rhythm of the class over time. A common mistake is to pack too much in. Clients need time to find each exercise, and you need time to look and correct. Aim for 3 to 5 minutes per exercise in the main block, with a mix of faster and slower work.

- **Watch the room, not the clock.** Time is a guide; the group is the reality.
- **Hold back material.** It is better to finish one exercise well than to rush through two.
- **Keep an easy exercise in reserve** to use if the group is tired, and a harder one if they are flying.

Transitions

Transitions are the moments between exercises. They can be the weakest part of a class or the best. A good transition is planned, short and part of the work.

- Plan the order so each exercise ends in the starting position of the next where possible.
- Cue the next position before the current exercise ends.
- Group exercises by position to avoid constant moving up and down.
- On the reformer, plan spring changes carefully and teach them in advance.
- Use breath in transitions: 'Exhale to roll up, inhale to find your seat.'

How to cue

A cue is any instruction that helps the client move. Strong cues are short, positive and specific. Use three types of cue and blend them according to the client.

TYPE	WHAT IT IS	BEST FOR	EXAMPLE
Verbal	Spoken instruction or image	Almost everyone; the basic tool	'Reach through the crown of your head.'
Tactile	Hands-on feedback, with consent	Clients who need to feel the area	A light touch on the ribs to cue breath
Visual	Demonstration or mirror	Visual learners, complex shapes	Showing the shape of a roll up

Verbal cueing

- Use second person and present tense: 'You lengthen your spine'.
- Use one cue at a time. If you give three, the client usually hears none.

- Say what to do, not what to avoid: 'Soften your ribs' rather than 'Don't flare'.
- Use imagery for feeling and anatomy for clarity. Mix both.
- Time cues with the breath: give the next instruction on the exhale or at a pause.

Tactile cueing with consent

Touch can be one of the most powerful tools you have, and one of the most risky if used carelessly. Always ask for consent before touching a client, and respect the answer.

1. Ask at the start of a class or session: 'Is it okay if I use light touch to help with positioning?'
2. Re-ask before each touch in a private session: 'May I place my hand on your ribs?'
3. Tell the client what you will do and where.
4. Use a light, flat hand rather than a poking finger.
5. Remove your hand as soon as the client has the feeling.
6. Accept 'no' without comment and offer another cue.

> **Consent is ongoing**
> A yes today is not a permanent yes. Some clients have a history that makes touch uncomfortable. Offer visual or verbal alternatives and never make touch a condition of participation.

Visual cueing

- Demonstrate from where the client can see you, ideally from the front or the side.
- Mirror the client: if you are facing them, use the opposite side when you say left and right, or say 'my left, your right' sparingly.
- Show only the shape needed; avoid advanced variations during beginner work.
- Use props, a skeleton, drawings or videos to explain complex ideas.
- Step back from the group to scan the room.

How to correct without frustrating

Corrections are an essential part of teaching, but done badly they can make clients feel judged and discouraged. Use a simple model that keeps the client learning.

1. **Notice what is working.** Start with something true and positive.
2. **Choose one correction.** Pick the fault that most affects safety or quality.
3. **Say what to do.** Make the instruction clear and doable.
4. **Offer an option.** A modification can often fix a fault without a lecture.

5. **Check the result.** Look again and acknowledge the change.
6. **Move on.** Do not hover over one client for the whole class.

INSTEAD OF	TRY
You're arching your back.	Let your ribs soften toward your hips.
Your shoulders are up.	Slide your shoulder blades toward your back pockets.
That's not right.	Try it again with your knees a little higher.
Don't strain your neck.	Let your head be heavy; use your hands if you like.
You need to engage your abs.	Draw your navel gently toward your spine as you exhale.

Voice and presence

- **Pace:** speak slower than you think. Pauses are powerful.
- **Volume:** project to the back of the room without shouting. Drop volume in the cool-down.
- **Tone:** warm, calm and confident. People follow a steady voice.
- **Language:** clear, simple and consistent. Use the same name for the same exercise every time.
- **Positioning:** move around the room. Do not hide behind a piece of equipment.
- **Energy:** match the energy of the class to the goal. Be lively in warm-up, focused in the middle and soothing at the end.
- **Humour and humanity:** a little goes a long way; never at a client's expense.

How to modify by level

Modifying is not dumbing down. It is shaping the exercise so that the client can feel its purpose safely. Use the following levers one at a time.

LEVER	EASIER	HARDER
Range	Smaller range of motion	Larger range
Lever length	Shorter lever, bent knees	Longer lever, straight legs
Base of support	Wider base, more contact	Narrower base, less contact
Tempo	Slower, with pauses	Faster or with controlled pulses
Load	Lighter springs, smaller prop	Heavier springs, bigger prop
Support	Hands or props to assist	Hands free

LEVER	EASIER	HARDER
Complexity	One limb at a time	Arms and legs together
Position	Supine or supported	Seated, standing or prone
Repetitions	Fewer reps or breaths	More reps or breaths

Change one thing at a time

If you change range, tempo and load together, the client cannot tell which change helped. Adjust one lever, check, then adjust another if needed.

Build Your Own Class in 5 Minutes

> *You do not have to rely on a pre-written class every time. With a small set of building blocks and a few rules, you can design a safe and varied 60-minute class in about five minutes.*

The method

1. **Pick the level and the equipment.** Who is in the room and what is available?
2. **Choose a focus.** One theme, such as spine mobility, hips and legs, shoulder stability or full-body flow.
3. **Fill the shape.** 10 minutes warm-up, 40 minutes main block, 10 minutes cool-down.
4. **Choose blocks by category.** Use the table below to select one or more exercises per category.
5. **Order for flow.** Group by position and alternate regions.
6. **Check the balance.** Make sure you have flexion, extension, rotation, lateral flexion and balance.
7. **Write your cues and modifications.** One line each for the key exercises.

Block table

BLOCK	CATEGORY	TIME	AIM	EXAMPLE CHOICES
1	breath	2 min	Arrive and settle	Lateral breathing, standing breath
2	warmup	3 min	Mobilize joints	Pelvic clock, cat and cow
3	spine	5 min	Articulate the spine	Roll down, spine stretch

BLOCK	CATEGORY	TIME	AIM	EXAMPLE CHOICES
4	abs	8 min	Build trunk stability	Hundred, single leg stretch
5	legs	7 min	Hips and legs	Side-lying leg series, footwork
6	arms	6 min	Shoulders and arms	Arm circles with band, push up
7	full	8 min	Integrate	Plank series, teaser variations
8	balance	6 min	Coordination and control	Standing balance, single-leg work
9	stretch	6 min	Lengthen and release	Hamstring stretch, spine stretch
10	cooldown	4 min	Settle	Breath, rest, closing cue

These ten blocks add up to 55 minutes; add 5 minutes of extra time for transitions and spoken instructions to reach a full hour.

An example, step by step

Say you have a beginner group, a mat and a band, and you want a class focused on the spine.

1. Level: beginner. Equipment: mat and band. Focus: spine mobility.
2. Breath (2 min): lateral breathing in supine.
3. Warm-up (3 min): pelvic clock, knee folds.
4. Spine (8 min): pelvic curl, cat and cow, spine stretch forward.
5. Abs (8 min): curl up, single leg stretch, with regressions.
6. Legs (6 min): bridge variations, side-lying leg lifts.
7. Arms (5 min): band rows, arm circles.
8. Full (6 min): plank on knees, swimming prep.
9. Stretch (6 min): hamstring stretch, mermaid stretch.
10. Cooldown (6 min): breath, rest and closing cue.
11. Total: 50 minutes of content plus about 10 minutes of transitions and explanations.

Rules for good class design

- Move from simple to complex and from supported to unsupported.
- Do not work the same muscle group in two exercises in a row if you can avoid it.
- Counter flexion with extension and extension with flexion.

- Place the most demanding balance work after the warm-up, not at the end when fatigue builds.
- Keep a regression and a progression ready for each key exercise.
- Finish with something calming even if the group is energetic.

CLASS DESIGN CHECKLIST

☐ Level and equipment confirmed.

☐ One clear focus.

☐ Warm-up, main block and cool-down in about the right proportions.

☐ Spine mobilized in more than one direction.

☐ Both upper and lower body worked.

☐ At least one balance or coordination challenge.

☐ A stretch that matches what was worked.

☐ Modifications noted for pregnancy, injury or older clients.

Modifications and Adaptations

> *Clients arrive with bodies that have histories. A good instructor adapts, with humility about what they do not know. This chapter gives conservative suggestions for common situations and clear guidance on when to refer to a professional.*

Educational information, not medical advice

The suggestions below are general education for instructors. They are not medical advice and do not replace the guidance of a client's physician, midwife, physical therapist or other healthcare professional. When in doubt, ask for clearance and follow it. Your role is to teach movement within your scope of practice, never to diagnose or treat.

Pregnancy

Many pregnant clients benefit from gentle movement, but every pregnancy is different. Always ask the client to confirm with their healthcare provider that exercise is appropriate, and follow their guidance. The suggestions below are intentionally conservative.

TRIMESTER	GENERAL CONSIDERATIONS	AVOID OR LIMIT	SUBSTITUTE WITH
First	Fatigue and nausea are common; energy varies day to day. Keep intensity moderate and let the client lead.	Overheating, breath holding, very vigorous work, extreme stretching	Gentle mobility, breath work, pelvic floor awareness, standing and side-lying work
Second	The belly grows, balance shifts; hormones increase joint laxity.	Lying on the back for long periods, lying prone, deep twists that compress the abdomen, repeated strong abdominal flexion, extreme ranges	Side-lying and seated work, inclined positions with wedge or bolster, standing balance with support, open twists from the upper back
Third	Weight and fatigue increase; balance is reduced; pelvic and back discomfort is common.	Staying on the back, prone positions, strong abdominal curls, jumping, fast changes of position, deep stretching	Seated and side-lying work, wall-supported standing, gentle pelvic tilts on all fours, breathing and relaxation

- Avoid lying prone (on the stomach) throughout pregnancy.
- From the second trimester, avoid long periods lying flat on the back; use a wedge or work in side-lying or inclined positions.
- Avoid repeated strong spinal flexion with heavy abdominal pressure; follow the healthcare provider's guidance.
- Watch for signs that should stop the session at once, such as dizziness, shortness of breath, pain, bleeding or fluid leaking, reduced fetal movement, severe headache or contractions, and advise the client to contact their provider.
- Keep the room cool, offer water and rest often.
- Be especially careful with balance work and with spring-based equipment when getting on and off.

> **When to refer**
>
> If a client reports bleeding, severe pain, dizziness that does not pass, a high-risk pregnancy diagnosis, or any symptom that worries you, stop and ask them to speak to their healthcare provider before continuing.

Older adults

Pilates can be a good fit for older adults when it is adapted to their needs. Priorities are balance, safety, strength for daily life and confidence. Many older clients have

conditions such as osteoporosis, arthritis or high blood pressure, so intake and clearance matter.

- **Avoid:** deep or loaded spinal flexion and forceful twisting when bone density is a concern, getting up and down from the floor quickly, rapid direction changes, and long breath holds.
- **Substitute:** seated and chair-based work, wall support for standing, supported bridging, extension work with care, and slow controlled movement.
- **Prioritize:** weight shifting, sit-to-stand patterns, hip strength, foot work, arm strength and posture.
- **Set up for safety:** a stable chair nearby, non-slip surfaces, room to move, and a clear route to the floor and back up.
- **Pace:** allow more time to change positions and more rest between exercises.
- **Refer:** ask for clearance with a diagnosed osteoporosis, heart condition, recent fall, dizziness, uncontrolled blood pressure or recent surgery.

Lower back injury or pain

Low back pain is common and has many causes. You are not there to diagnose it. The aim is to teach safe movement that does not aggravate symptoms and, with the provider's clearance, to build general strength and awareness.

AVOID	SUBSTITUTE	NOTES
Repeated or heavy spinal flexion, such as full roll ups and rolling like a ball, if flexion aggravates symptoms	Pelvic tilts, bridging, small curl ups, supported positions	Let comfort guide range; if pain increases, stop
Deep extension with compression, such as full swan	Prone prep with small range, quadruped extension, sphinx on forearms	Only if extension is comfortable
Loaded twisting from the lumbar spine	Gentle thoracic rotation in sitting or lying	Rotate from the ribs, not the low back
Double leg lowers with the back lifting	Single-leg work, feet on the floor, bent knees	Keep the lower back quiet
Fast, ballistic or high-load work	Slow, controlled movement and lighter springs	Choose control over range

- Ask which movements feel better and which feel worse, and let that guide your choices.
- Teach neutral spine and gentle deep abdominal activation as basic skills.

- Keep the hips and thoracic spine mobile so the lower back does not have to compensate.
- Stop any movement that sends pain down a leg, causes numbness, tingling or weakness, and advise the client to seek assessment.

> **When to refer**
>
> Refer or ask for clearance for pain after a fall or accident, numbness, weakness in a leg, loss of bladder or bowel control, unexplained weight loss with back pain, night pain, fever or pain that is worsening. Some of these need urgent medical attention.

Knee issues

AVOID	SUBSTITUTE	NOTES
Deep knee bends with loaded flexion, deep lunges	Partial range squats, supported wall squats, bridging	Stay in a pain-free range
Twisting on a planted foot	Keep feet pointing the same way as the knees, pivot with the whole body	Protect the joint from torque
Full kneeling with pressure on the kneecap	Use extra padding or choose a different position	Pad the knees
Hyperextension or locked-out knees	Keep a soft bend	Cue the thighs to be active
High-impact or jumping work	Low-impact footwork, light spring work	Reintroduce load slowly

- Check foot and hip alignment first; knee issues are often influenced by both.
- Strengthen the hips and the muscles around the thigh, as comfort allows.
- Reduce range and load before removing exercises entirely.
- Stop if there is swelling, locking, giving way, or sharp pain.

Shoulder issues

AVOID	SUBSTITUTE	NOTES
Weight-bearing in full plank or push up if painful	Plank on knees, wall push up, incline plank	Reduce load first
Overhead reaching with load if painful	Reaching in a smaller range, arms in front	Work below shoulder height
Pulling the arms far behind the body	Smaller range, arms by the side	Protect the front of the shoulder

AVOID	SUBSTITUTE	NOTES
Hanging or heavy arm spring work	Lighter springs, shorter range, supported positions	Stabilize the shoulder blade first
Rolling over onto the neck and shoulders	Bridge or hip lifts instead	Keep weight off the neck

- Cue the shoulder blades to rest wide and heavy before any arm movement.
- Use the serratus and lower trapezius with gentle push-pull and wall work.
- Begin with arms supported, then progress to unsupported.
- Stop if the client reports pinching, catching, weakness or pain at night, and refer to a professional.

Quick reference for referral

SITUATION	ACTION
Pregnancy	Ask for provider clearance; follow conservative guidelines; stop at any warning sign
Recent surgery or injury	Request written clearance and follow restrictions
Osteoporosis or osteopenia	Request guidance; avoid loaded flexion and forceful twisting
Heart or blood pressure condition	Request clearance; avoid breath holding
Neurological symptoms	Stop and refer
Chest pain, severe dizziness, fainting	Stop immediately and call for help
Any pain that worsens during exercise	Stop that exercise and ask the client to see a professional

BEFORE ADAPTING FOR A SPECIAL POPULATION

☐ Intake form completed and read.

☐ Healthcare provider clearance requested where appropriate.

☐ Stated limits and avoid-lists written in the client record.

☐ Modification chosen for each exercise in advance.

☐ Client told they can stop or change anything at any time.

☐ Plan for when to refer.

Working with Different Clients

> *The same exercise can be taught in many ways. Knowing who is in front of you helps you choose the right level, language and pace.*

Beginners

- **Goals:** learn the basics, build confidence, feel the breath and the centre.
- **Approach:** teach fewer exercises more thoroughly. Keep language simple. Avoid jargon.
- **Choose:** supine and side-lying positions, small ranges, props for support.
- **Watch for:** breath holding, comparing themselves to others, trying too hard.
- **Encourage:** curiosity and patience. Tell them it takes time.

Athletes

- **Goals:** improve control, balance, mobility and body awareness; complement training and help recovery.
- **Approach:** be specific. Explain how the exercise links to their sport. Respect their competitive drive but keep control as the focus.
- **Choose:** single-leg work, rotation, deceleration, hip and shoulder stability, and breath-controlled endurance.
- **Watch for:** momentum, over-reliance on strong muscles, ignoring pain.
- **Encourage:** quality before quantity; remind them that slow is hard.

Post-natal clients

After childbirth, the body goes through many changes. Return to exercise should follow the healthcare provider's clearance, often given at a postpartum check-up, and may differ after a caesarean birth or complications.

- **Start with:** breathing, gentle pelvic floor awareness, posture for feeding and carrying, and walking.
- **Progress gradually:** from lying and seated work to standing and loaded exercises.
- **Be mindful of:** abdominal separation (diastasis recti). Avoid exercises that visibly dome or cone the midline, and refer to a women's health physical therapist for assessment.
- **Also consider:** pelvic floor symptoms such as leaking or heaviness, which are a reason to refer rather than push through.
- **Support the whole person:** sleep, stress and time are limited. Offer short, achievable sessions.

- **Check in:** ask about caesarean scars, pain and bleeding at each session.

> **Post-natal clearance**
>
> Do not begin a post-natal client on core-focused work until they have received clearance from their provider. Be especially cautious after caesarean birth and in the first months postpartum.

Clients aged 50 and over

- **Goals:** maintain strength and mobility, protect bone and joint health, improve balance, and enjoy movement.
- **Approach:** respect experience and history; avoid patronizing language. Treat every person as capable.
- **Choose:** standing balance work, hip and leg strength, posture and shoulder mobility, and spinal extension work in a comfortable range.
- **Adjust:** allow more time, more rest, and more support.
- **Watch for:** breath holding with effort, dizziness on position changes, joint stiffness in the morning, and reluctance to mention pain.
- **Encourage:** consistency over intensity; celebrate everyday wins such as easier stairs.

Working with groups of mixed levels

- Teach the base version and offer one regression and one progression for each exercise.
- Say each option aloud so no one has to ask.
- Use phrases such as 'if you want more' and 'if you want to ease off' rather than levels.
- Walk the room and give individual cues quietly.
- Group clients with similar needs when the room layout allows.

CLIENT	PRIMARY FOCUS	COMMON CUE STYLE	WATCH FOR
Beginner	Breath, centre, basics	Simple and image-based	Holding the breath
Athlete	Control, mobility, balance	Specific and technical	Momentum
Post-natal	Gentle return, pelvic floor, posture	Calm and encouraging	Doming, leaking, fatigue

CLIENT	PRIMARY FOCUS	COMMON CUE STYLE	WATCH FOR
50 and over	Balance, bone and joint health	Clear and respectful	Dizziness, stiffness
Mixed group	Inclusion	Options for all	Someone being left behind

Safety, Scope of Practice and Screening

> *Safe teaching starts before the first exercise. It begins with knowing what you may and may not do, asking the right questions and building a habit of care.*

Scope of practice

A Pilates instructor teaches movement. They are not a doctor, physical therapist, dietitian or psychologist, even if they have deep knowledge in those areas. Staying within your scope protects your clients and you.

YOU CAN	YOU SHOULD NOT
Teach exercises and give general movement guidance	Diagnose injuries or medical conditions
Adapt exercises for stated limitations	Promise to cure, treat or heal any condition
Encourage clients to see a professional	Tell a client to stop or change medication
Share general information about exercise and posture	Give nutrition plans or prescribe supplements
Follow the guidance of a client's healthcare team	Contradict a healthcare provider's advice
Keep records of what you teach and observe	Share a client's information without consent

Screening questions

Before any new client begins, collect basic health information. A simple questionnaire in the style of a physical activity readiness questionnaire helps you identify who may need clearance. It does not replace medical advice and it does not make you responsible for diagnosing anything.

> **Not a medical tool**
>
> These sample questions are a starting point for conversation and recordkeeping. They do not replace a medical examination or the guidance of a healthcare provider. If any answer raises a concern, ask the client to consult their provider before beginning.

1. Has a doctor ever told you that you have a heart condition or high blood pressure?
2. Do you ever feel pain in your chest when you are physically active?
3. Do you ever lose your balance because of dizziness, or have you lost consciousness?
4. Do you have a bone, joint or soft tissue problem, including back, knee or shoulder, that could be made worse by exercise?
5. Are you currently pregnant, or have you given birth in the last twelve months?
6. Have you had surgery in the last twelve months?
7. Are you taking any medication that may affect how you exercise?
8. Do you have osteoporosis, osteopenia or a family history of it?
9. Do you have any other medical conditions or reasons you should not exercise?
10. Is there anything else you would like me to know to keep your sessions safe and enjoyable?

If a client answers yes to any item, ask for details, record them, and, when appropriate, request written clearance from their healthcare provider. Keep the form on file and review it at regular intervals, as health changes.

Dos and don'ts in the studio

DO	DON'T
Ask about injuries and limits at every session	Assume nothing has changed since the last visit
Teach the breath first	Rush into complex exercises
Use the lightest option that works	Push clients past comfortable effort
Say 'stop if it hurts'	Say 'push through the pain'
Stay present and watch the room	Look at your phone or notes too long
Keep records and notes	Rely on memory
Check the equipment before each class	Use broken or worn equipment
Know your emergency plan	Hope nothing will happen

Emergency basics

- Know where the first aid kit, the phone and the exit are.
- Hold a current first aid and CPR certification if possible.
- If a client feels faint, have them sit or lie down, keep them calm, and call for help if they do not recover quickly.
- For chest pain, trouble breathing, a suspected fracture or loss of consciousness, call emergency services at once.
- Write down what happened and what you did as soon as you can.

Professional habits

- Keep your own training up to date and attend continuing education.
- Know your limits and be honest about them.
- Respect privacy; keep client information confidential.
- Maintain clear boundaries about touch, time and communication.
- Carry appropriate liability insurance as required where you work.
- Build relationships with local physical therapists, midwives and doctors, so referral is easy.

NEW CLIENT SCREENING CHECKLIST

☐ Health history form completed and reviewed.

☐ Pregnancy, injury and surgery questions asked.

☐ Clearance requested where needed.

☐ Goals and expectations discussed.

☐ Consent for touch discussed.

☐ Emergency contact on file.

☐ Plan for a first session agreed.

Glossary

> *Pilates has its own language. Use this reference to speak clearly with colleagues, clients and healthcare professionals, and to explain terms in plain words.*

TERM	MEANING
Abduction	Moving a limb away from the midline of the body.
Adduction	Moving a limb toward the midline of the body.

TERM	MEANING
Anterior pelvic tilt	The front of the pelvis tips down, increasing the lower back curve.
Articulation	Moving the spine one segment at a time, like a chain.
Barrel	A curved apparatus such as the Ladder Barrel or Spine Corrector.
Bridge	Lifting the hips from a lying position with bent knees.
Cadillac	A raised table with a frame, bars and springs; also called the Trapeze Table.
Carriage	The sliding platform on a reformer.
Centre	The area from the lower ribs to the pelvis; also called the powerhouse.
Cervical spine	The seven vertebrae of the neck.
Concentric	A muscle action in which the muscle shortens under load.
Contraindication	A condition or situation in which an exercise should be avoided.
Core	The group of muscles that stabilize the trunk and pelvis.
Cue	A short instruction that guides movement or awareness.
Diaphragm	The dome-shaped breathing muscle under the lungs.
Diastasis recti	A widening of the gap between the two sides of the rectus abdominis.
Dosage	The amount of an exercise: reps, seconds, breaths and sets.
Eccentric	A muscle action in which the muscle lengthens under load.
Extension	Straightening or arching; moving a joint to open its angle.
Flexion	Bending or rounding; moving a joint to close its angle.
Flow	Smooth, continuous movement from one exercise or phase to the next.
Footbar	The bar on a reformer used for the feet or hands.
Footwork	The classic opening series on the reformer performed with the feet on the bar.
Hamstrings	The muscles at the back of the thigh.
Hip flexors	Muscles at the front of the hip that lift the thigh.
Hollowing	Gently drawing the lower abdomen in toward the spine.

TERM	MEANING
Hyperextension	Extending a joint beyond its comfortable or normal range.
Imprint	A small posterior tilt of the pelvis that brings the lower back toward the mat.
Isometric	A muscle action in which the muscle works without changing length.
Kyphosis	A forward rounding of the upper back.
Lateral flexion	Bending the spine to the side.
Lateral thoracic breathing	Breathing that expands the ribcage sideways and back.
Lordosis	The inward curve of the lower back or neck.
Lumbar spine	The five vertebrae of the lower back.
Magic Circle	A flexible ring used for light resistance.
Modification	A change to an exercise to make it easier, safer or more suitable.
Neutral pelvis	A level pelvis between a full arch and a flat back.
Neutral spine	The position in which the spine keeps its natural curves.
Obliques	The muscles on the sides of the abdomen that rotate and side-bend the trunk.
Pelvic clock	An imagery exercise rolling the pelvis through the numbers of a clock.
Pelvic floor	The group of muscles that form the base of the pelvis.
Posterior pelvic tilt	The front of the pelvis tips up, flattening the lower back.
Powerhouse	The traditional name for the centre of the body.
Progression	A harder version of an exercise.
Prone	Lying face down.
Props	Small tools such as balls, bands and rings.
Quadriceps	The muscles at the front of the thigh.
Quadruped	On hands and knees.
Range of motion	The distance a joint can move.
Regression	An easier version of an exercise.

TERM	MEANING
Rep	One complete performance of an exercise.
Ribcage	The bony cage of ribs protecting the lungs and heart.
Rotator cuff	Four small muscles that stabilize the shoulder.
Sacroiliac joint	The joint connecting the sacrum and the hip bone.
Scapula	The shoulder blade.
Scapular stability	Control of the shoulder blade on the ribcage.
Side-lying	Lying on one side.
Spine	The column of vertebrae from the skull to the tailbone.
Spotting	Standing close to a client to help prevent falls or errors.
Springs	Metal coils that provide resistance on spring-based equipment.
Supine	Lying face up.
Tabletop	Legs lifted with knees bent at 90 degrees over the hips.
Tactile cue	A cue given by touch, with consent.
Thoracic spine	The twelve vertebrae of the upper and mid back.
Transversus abdominis	The deepest abdominal muscle, wrapping around the trunk.
Warm-up	The first part of a class that prepares the body for work.
Wunda Chair	A compact apparatus with a pedal and seat.

The Cue Bank

> *A cue is a short sentence you can say aloud at the right moment. This bank holds more than 150 cues grouped by region. Use them as written, mix them, or shape them into your own voice. The best cue is the one that makes the client change, so test, listen and adapt.*

How to use the bank

Pick one cue per correction. Say it on the exhale or in a pause. If it does not work after two tries, try a different cue, a different type (image, anatomy or touch) or a different version of the exercise.

Breath

- Breathe into the sides of your ribs.
- Let your ribs widen like an umbrella opening.
- Inhale to prepare, exhale to move.
- Exhale like you are fogging a mirror.
- Let the breath be long and quiet.
- Breathe into your back ribs.
- Soften your belly as you inhale.
- Draw your lower abdomen in as you exhale.
- Let the exhale empty you completely.
- Breathe low and wide, not high and tight.
- Keep breathing; do not hold anything.
- Let the breath carry the movement.
- Count your breaths, not your reps.
- Feel your ribs close like a fan.
- Let each exhale melt your shoulders down.

Pelvis and lower back

- Find a neutral pelvis.
- Let your lower back melt toward the mat.
- Tip your pelvis like a bowl of water.
- Imagine a clock face on your belly.
- Roll your pelvis toward twelve o'clock.
- Keep your pelvis level like a tray.
- Lengthen your tailbone away from your ribs.
- Let your sitting bones widen.
- Keep your hips quiet while your legs move.
- Draw your hip bones toward each other.
- Soften your lower back.
- Let your pelvis stay heavy.
- Stack your hips over your knees.
- Keep both hip bones pointing to the ceiling.
- Hold your pelvis still like a table.

Spine

- Lengthen your spine.
- Reach through the crown of your head.
- Peel your spine off the mat one bone at a time.
- Roll down like you are rolling down a wall.
- Stack your spine, one vertebra at a time.
- Grow taller as you exhale.
- Make your spine long like a rope.
- Open the space between each vertebra.
- Imagine a string pulling the top of your head to the ceiling.
- Curve over a big beach ball.
- Rotate from your ribs, not your lower back.
- Keep your spine long as you twist.
- Reach your breastbone forward and up.
- Lift your chest without gripping your back.
- Smooth the curve of your spine like a wheel.

Shoulders and neck

- Slide your shoulder blades into your back pockets.
- Widen across your collarbones.
- Let your shoulders melt away from your ears.
- Keep your neck long.
- Float your chin, as if holding a peach under it.
- Let your head be heavy.
- Keep your gaze soft and forward.
- Soften your jaw.
- Keep your shoulder blades wide and heavy.
- Draw your shoulder blades down your back.
- Keep your throat relaxed.
- Nod gently, keeping the back of your neck long.
- Let your upper trapezius quiet down.
- Lift only as high as your neck allows.
- Keep the base of your neck soft.

Arms

- Reach long through your fingertips.
- Keep your elbows soft.
- Press the air away with your palms.
- Draw a long circle with your arms.
- Let your arms float.
- Reach your arms like wings.
- Keep your elbows pointing back.
- Squeeze a small ball between your elbows.
- Keep your wrists long.
- Reach through your middle fingers.
- Open your arms like a book.
- Press your forearms into the mat.
- Keep your arms heavy and wide.
- Pull your elbows to your ribs.
- Let your arms move from your back.

Legs

- Reach long through your legs.
- Keep your knees in line with your toes.
- Press your thighs together.
- Lengthen from your hip to your heel.
- Soften your knees.
- Let the leg move from the hip, not the knee.
- Draw your knee toward your chest.
- Keep your thighs rotating slightly out.
- Squeeze your inner thighs.
- Lift your leg without tipping your pelvis.
- Keep your hips stacked.
- Press your heels together.
- Imagine your legs are one long line.
- Keep your legs heavy and long.
- Move your leg like it is floating in water.

Feet

- Spread your toes wide.
- Press through three points of your foot.
- Press through your heels.
- Keep your arches lifted.
- Point through the long line of your foot.
- Flex your feet and reach through your heels.
- Ground your big toe.
- Keep your weight even across your feet.
- Imagine you are standing on sand.
- Grip the floor with your toes, softly.
- Roll through your feet.
- Feel your feet like roots.
- Keep your ankles soft.
- Press the ball of your foot into the bar.
- Lengthen your toes away from your ankles.

Core

- Draw your navel to your spine.
- Engage your deep abdominals gently.
- Zip up from your pelvis to your ribs.
- Wrap your waist like a corset.
- Hollow your belly like a scoop.
- Imagine hugging your waist in toward your spine.
- Lift your pelvic floor like an elevator.
- Keep your ribs knitting together.
- Gently narrow your waist.
- Keep 30 percent effort in your abdomen.
- Let your ribs sink toward your hips.
- Draw your lower belly up and in.
- Connect your ribs to your hips.
- Feel the center of your body working.
- Brace lightly, as if for a gentle laugh.

Imagery

- Imagine your spine is a string of pearls.
- Imagine your head floating on a balloon.
- Imagine you are sliding between two panes of glass.
- Imagine your body is a long, straight pencil.
- Imagine you are a rolling wheel.
- Imagine pouring water down your spine.
- Imagine your ribs are a bellows.
- Imagine you are lengthening on a rack.
- Imagine you are pressing into warm sand.
- Imagine your chest is a window opening.
- Imagine a pencil drawing a line from your tailbone.
- Imagine your arms resting on water.
- Imagine your pelvis is a bowl of soup.
- Imagine you are pulled up by a thread.
- Imagine you are melting into the mat.

Flow and finishing

- Move with the breath.
- Slow it down.
- Make it smooth, not fast.
- Finish what you start.
- Smooth the transitions.
- Quality over quantity.
- Move as if through honey.
- Pause and notice.
- Let it be easy.
- Keep it light and controlled.
- Take your time.
- Use less effort than you think.
- Make every rep the same.
- End with control.
- Stay with the movement.
- Listen to your body.

- Let the movement breathe.
- Keep it simple.
- Begin again with the breath.

Many of these cues work across regions. A breath cue can help a leg exercise and a spine cue can help a shoulder problem. Keep the bank where you can scan it quickly and add your own cues as you teach.

Next Step

> *You now hold the whole Library: 500 exercises, 100 classes and the manual that ties them together. The next step is simple and it is yours to take.*

1. **Read this manual once, end to end.** Mark the pages you will return to.
2. **Choose a class and teach it.** Start with a beginner class you know well and teach it exactly as written.
3. **Write down what worked.** Note the cues that landed and the transitions that stumbled.
4. **Adapt one block.** Use the five-minute method to swap in an exercise of your own.
5. **Keep the Cue Bank open.** Add one new cue each week.
6. **Practice what you teach.** The best instructors move every week.

Teaching is a skill built through repetition and reflection. Every class gives you data. Use it with curiosity and kindness toward your clients and yourself.

> **Remember**
>
> This library is educational. Always work within your scope, respect your clients' health information and refer to healthcare professionals when in doubt. Safe teaching is good teaching.